BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative stages explain the order of care; they are not equally spaced appointments. FBC, kidney and liver profile at baseline and before every 21-day cycle; review symptoms, neuropathy, fluid retention and the steroid plan. Confirm remaining cycles against the six-cycle NSCLC maximum.

Check	Before starting	Early treatment	Ongoing treatment	Pause / dose change	Completion / next decision
Blood / kidney / liver	Baseline FBC, kidney and liver tests	Before each 21-day cycle	Repeat before each planned dose	Check recovery before restarting	Follow unresolved abnormalities
Symptoms / premedication	Review symptoms, medicines and reaction history	Check steroid doses, swelling and neuropathy	Review before every treatment	Record worst and current symptoms; clinician confirms changes	Review persistent symptoms and recovery
Heart / breathing	Assess history and symptoms	Infusion observations and symptom review	Extra tests if symptoms or risk require	Urgent assessment for chest pain, fainting or new breathlessness	Agree follow-up for unresolved symptoms
Cancer / course	Confirm 00203a NSCLC after prior chemotherapy	Agree response review and actual appointment plan	Up to six cycles; stop earlier if needed	Reassess benefit, toxicity and priorities	Agree next treatment or supportive-care plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00203 v7 · official regimen · EMA Taxotere product information; retrieved 29 October 2026

THE WHOLE TREATMENT JOURNEY

Docetaxel 75 mg/m² · previously treated NSCLC: treatment and checks

Lung · previously treated advanced NSCLC

Docetaxel 75 mg/m² is infused on Day 1 every 21 days. For this NSCLC indication, the NCCP plan has a maximum of six cycles; treatment stops earlier for progression or unacceptable side effects. Your team confirms your prescribed dose and course.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Pause / dose change	Completion / next decision
Treatment plan NCCP 00203	Confirm indication and prescription	Day 1 every 21 days	Maximum six cycles for NSCLC	Clinician review before restart	Agree the next plan
Blood / kidney / liver NCCP 00203 · SMPC	Baseline FBC, kidney and liver tests	Before each 21-day cycle	Repeat before each planned dose	Check recovery before restarting	Follow unresolved abnormalities
Symptoms / premedication NCCP 00203 · SMPC	Review symptoms, medicines and reaction history	Check steroid doses, swelling and neuropathy	Review before every treatment	Record worst and current symptoms; clinician confirms changes	Review persistent symptoms and recovery
Heart / breathing NCCP 00203 · SMPC	Assess history and symptoms	Infusion observations and symptom review	Extra tests if symptoms or risk require	Urgent assessment for chest pain, fainting or new breathlessness	Agree follow-up for unresolved symptoms
Cancer / course NCCP 00203 · SMPC	Confirm 00203a NSCLC after prior chemotherapy	Agree response review and actual appointment plan	Up to six cycles; stop earlier if needed	Reassess benefit, toxicity and priorities	Agree next treatment or supportive-care plan

What each check is for

1 Safety review

Before each 21-day cycle

FBC, kidney and liver profile at baseline and before every 21-day cycle; review symptoms, neuropathy, fluid retention and the steroid plan. Confirm remaining cycles against the six-cycle NSCLC maximum.

Ask / act: The team reviews results, current symptoms and the prescription.

Sources: NCCP 00203 · SMPC

2 Benefit and treatment choice

At consultant-agreed response reviews

A historical randomised trial found that docetaxel 75 mg/m² every three weeks prolonged survival compared with supportive care alone in selected people whose NSCLC had progressed after platinum chemotherapy. Modern previous treatment and available alternatives differ, so the trial cannot predict an individual outcome.

Ask / act: Discuss continuing, changing or stopping treatment according to benefit, toxicity and priorities.

Sources: NCCP 00203 · TRIAL

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.25